



CLINICAL PRACTICE GUIDELINE	
Applying the Mental Health Act 2014 in WNHS	
Scope (Staff):	Health professionals caring for women with mental illness, including mental health staff working in the Mother and Baby Unit and the Department of Psychological Medicine
Scope (Area):	Women's Mental Health

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1. Aim

The purpose of this guideline is to ensure all Women's Health, Genetics and Mental Health (WHGMH) Directorate mental health staff provide care for women, their babies and families in compliance with legislative requirements and have access to information related to [Mental Health Act 2014](#), [Mental Health Regulations 2015](#) and the Chief Psychiatrist's [Standards](#) (2015) and [Guidelines](#) (2015).

For non-mental health clinicians at King Edward Memorial Hospital (KEMH) further information can be found in the [Mental Health Policy in Women and Newborn Health Service](#).

2. Risk

Non-compliance with this policy will:

Breach legislative requirements	☒	Impact on Patient Quality of Care	☒
Breach National/State/Mandatory Policy	☒	Impact on Patient Safety	☒
Breach professional standards	☒	Impact financial operations	☐
Damage NMHS reputation	☒	Impact on Staff Safety	☒
Other	☐		

3. Principles

- WHGMH will ensure psychiatrists are appropriately trained before administering the Mental Health Act 2014 (MHA 2014) and meet the definition of a psychiatrist under the MHA 2014 – see [Office of Chief Psychiatrist website](#).
- All mental health clinical staff must undertake the MHA 2014 Clinicians' eLearning package:
 - [Office of Chief Psychiatrist - education and training page](#)
 - [MHA 2014 eLearning package](#)
 - Access Mental Health Commission (MHC) MHA 2014 information brochures here: [MHC WA - MHA 2014 Resources](#)
 - The only accredited WNHS mental health beds are in the KEMH MBU. External psychiatrists do not have admitting rights to the MBU.

4. Compliance and Evaluation

WHGMH Directorate mental health staff are responsible for complying with this policy. The Head of Department, Mother Baby Unit/ Director of Psychiatry and the Nurse Co-Director of the WHGMH Directorate may request quantitative data and other compliance evidence information from the Mother Baby Unit and the Department of Psychological Medicine in relation to the requirements of this policy. In these cases, management will work together to agree on the information and timeframes within which this is to be provided.

5. Mental Health Act 2014 Forms

- MHA 2014 Forms must be completed via **PSOLIS** in the first instance, this will ensure that appropriate notifications are sent automatically.
- If access to PSOLIS is not available then a paper copy is to be completed and transcribed into PSOLIS at a later date (relevant notifications are listed on back of Forms).
- Hard copies of MHA 2014 Forms can be found in the back of KEMH Emergency Mental Health File (pink file) located in Triage Office in Psychological Medicine, Emergency Centre, Hospital Clinical Manager's

Office, Maternal Fetal Assessment Unit, Labour and Birth Suite, Sexual Assault Resource Centre and MBU; the file is also available [electronically](#).

- Alternatively, the MHA 2014 Forms can be downloaded from the DOH [Mental Health Unit website](#) or the [Office of Chief Psychiatrist website](#) – the latter includes both writable and printable forms with instructions related to when to use the form, duration of order, relevant criteria and related checklist as well as required notifications.
- Use PSOLIS print function to store a hard copy in patient's medical record.
- For support to complete MHA 2014 Forms on PSOLIS please contact PSOLIS Support on **1300 170 089**

6. Clinicians' Practice Guide to the Mental Health Act 2014

- Staff will have access to the [Clinicians' Practice Guide to the Mental Health Act 2014 \(3rd Edition\)](#) (CPG) produced by the Office of the Chief Psychiatrist. The document is a legislative guide written for mental health clinicians and other health staff who may need to use the MHA 2014 when providing treatment and care.
- For any queries which are not answered by the legislation or the Clinicians' Practice Guide consult with appropriate Clinical Lead in the first instance, who can then escalate to Chief Psychiatrist Delegate
- Relevant sections of the Clinicians' Practice Guide (CPG) include:

Absent without leave – see CPG 4.10 - page 100 - 102

- In the first instance, a thorough search of the KEMH MBU and surrounding environment will be conducted, including contact with the patient's partner/support person to establish their whereabouts.
- Once it is confirmed the patient is absent without leave and risk assessment completed, the Hospital Security is to be contacted on **Ext 86444** (or speed dial ***41101**) and the Hospital Clinical Managers **Ext 86146**. Security are to be provided with patient description details, a photograph of the missing patient and nursing staff are to request a thorough hospital and ground search.
- The Shift Coordinator or Clinical Nurse Specialist/Manager will contact Police, at Police Operations on **131444**, stating patient's legal status and degree of risk.
- If a patient absconds with a baby Child Protection and Family Support Crisis Care must be informed on **9223 1111**.

Further information:

Mandatory Policy (MP) 0012/16 [Missing Person Policy](#)

Office of Chief Psychiatrist – [Reporting notifiable incidents](#)

Delegations – See CPG 11.9 page 181.

In WNHS, the delegated powers of the Chief Psychiatrist are vested in the Head of Department, Mother Baby Unit/ Director of Psychiatry (or in their absence - Acting Head of Department, Mother Baby Unit/ Director of Psychiatry).

Further information:

Office of Chief Psychiatrist website [Process and Instruments of Delegation MHA 2014](#):

[Approval for Inpatient Treatment Order in General Hospital](#)

[Change in Supervising Psychiatrist for a Community Treatment Order](#)

[Approval for emergency electroconvulsive treatment.](#)

Emergency treatment for referred persons – See CPG 3.9 page 81

- Emergency Psychiatric Treatment can be provided to a patient (without informed consent) if it is essential to save the patient's life or prevent serious harm to themselves, their baby/ies or another person/s.
- Also see the "[KEMH Emergency Mental Health File](#)" (Pink file) for further resources – useful contact numbers, link to NMHS Mental Health Transport hub, Acute Health-related Behavioural Disturbance flowchart (inpatients), KEMH mental health care pathway flowcharts, location and list of emergency psychiatric drugs and Mental Health Act Forms.
- Also see WHGMMH Guideline – "[Mental Health Emergency, Management of](#)" – contains Acute Health-related Behavioural Disturbance flowcharts, principles of physical restraint in pregnancy and postpartum and delirium guideline.

Freedom and restriction of communication – See CPG 1.2 page 33

- When reviewing whether the restriction is to continue (confirmed), be revoked or allowed to expire, it is important the decision is recorded on PSOLIS.
- Even though the Form 12C restriction will automatically expire after 24 hours, it will remain listed on PSOLIS until updated by the clinician.
- If a decision to continue the restriction is recorded in the Form 12C log on PSOLIS before expiry, this will extend the Form 12C for another 24hrs.

Further opinion in regard to treatment – See CPG 1.13 page 35 - 40

When identifying an appropriate psychiatrist for a further opinion, it is important that a psychiatrist from the specialist perinatal field is considered. The following table sets out the options and timeframes for further opinions

Type of Further Opinion	Timeframe	KPI (% within timeframe)
Psychiatrist within Women and Newborn Health Service (WNHS) site	Within three (3) working days	80% within 3 days
Psychiatrist within North Metropolitan Health Service (NMHS) but outside WHNS	Within five (5) working days	80% within 5 days
Psychiatrist outside NMHS (different area health service – consider Fiona Stanley Hospital Mother Baby Unit)	Within five (5) working days	80% within 5 days
Private Psychiatrist at	As soon as practicable	As soon as

patient's own cost		practicable
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Further information:

Office of Chief Psychiatrist website – [Refusal of further opinion request by psychiatrist](#)

Chief Psychiatrist's guidelines – [Guideline \(c\) independence of psychiatrists and further opinions](#)

Mental Health Commission website – [MHA 2014 Further Opinions Consumer fact sheet and flowchart](#)

Mental Health Tribunal – See CPG page 152; Clinicians responsibilities page 157.

The MBU has a responsibility on an ongoing basis to indicate restricted entries in the patient's medical record by using the red stickers made available for this purpose. Also see CPG1.6.17 page 29 re: disclosing restricted information.

Further information:

Mental Health Tribunal - reporting templates and resources can be found at the following sites:

[Mental Health Tribunal - For medical reports](#)

[Mental Health Tribunal - Applications by doctors](#)

[Mental Health Tribunal - Preparing for hearings](#)

Mental Health Tribunal Hearing: Important information for Treating Teams – [Treating team instruction sheet](#)

Notifications

All [MHA 2014 Forms](#) requiring notifications include relevant information on the second page.

- If documenting into PSOLIS, notifications are forwarded automatically.
- For support in completing Forms on PSOLIS please contact PSOLIS Support on 1300 170 089.
- Notification of certain events (carers and families) CPG 1.24 pages 53 – 56
 - Every effort to inform carers/family members must be made inclusive of videoconference (via telehealth centre), or teleconference particularly for rural and/or remote support persons.
 - Notification sticker to be placed in the margin of the patient's medical record
- Receipt and review of notifiable incidents CPG 11.6 page 179, 180.
Chief Psychiatrist website:
 - [Policy for mandatory reporting of notifiable incidents](#) to Chief Psychiatrist (2018)
 - [Reporting Checklist](#) (2018) – Table 1. Overview of how to report to the Chief Psychiatrist
 - Also see [Reporting notifiable incidents – public mental health services](#)

- Chief Mental Health Advocate - Functions of the Chief Advocate including when to notify - CPD 8.2.4 page 145
Mental Health Advocacy Service - Freecall 1800 999 057
- Mental Health Tribunal CPG 9.2.4 page 153

Further information:

Mental Health Commission consumer publication – [List of notifiable events](#)

Mental Health Act 2014 Requirements for Aboriginal and Torres Strait Islander People¹

WNHS recognises, respects and values Aboriginal culture and ensures that any intervention with Aboriginal patients and their family does not diminish their cultural rights, expectations or practices. The DOH MP 0065/17 Aboriginal Cultural eLearning Policy aims to build the cultural knowledge of the WA health system workforce and strengthen its capacity to provide culturally respectful and non-discriminatory healthcare. In addition, staff must make every effort to comply with the Charter of Mental Health Care Principles to ensure the rights of Aboriginal women, infants and family members are met (principle 7).

Patients Under 18 Years CPG 1.23 pages 52 - 53

- Patients under 18 years may present to, or be a patient of WNHS for either maternity or gynaecological care with a pre-existing mental health condition and/or develop a mental health condition while an inpatient/outpatient. This may occur in Emergency Centre, Maternal Fetal Assessment Unit or general wards/departments/clinics.
- For adolescent mothers and their infants, depending on individual patient circumstances and if they meet the referral criteria, care may be appropriately provided at the MBU. If they do not meet the criteria or a more secure or intensive nursing environment is required, then refer to a youth facility:
 - Child and Adolescent Health Services –Perth Children's Hospital [Acute Mental Health and Inpatient Unit](#). These services provide tier 3 and 4 specialised statewide services to children aged 0-15 years and in some circumstances up to 17 years of age.
 - South Metropolitan Health Service – Fiona Stanley Hospital [Mental Health Youth Unit](#). These services provide care and treatment of young people aged 16 – 24 years.
- See CPG 1.23.12 – parents to be provided with a report (copy to Chief Psychiatrist)
[Segregation of children from adult inpatients](#) – notification form

For patients under 18 years detained in an authorised mental health unit ([Form 6A](#)) the following applies:

¹ In Western Australia the term 'Aboriginal' is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to the Torres Strait Islander community.

- Following examination the psychiatrist may make an involuntary treatment order (ITO) authorising detention at an authorised hospital for up to 14 days for a young person.
- Timeframe to notify third parties, including a carer, close family member or other personal support person, of order: **within 2 hours**.
- Timeframe to notify third parties of revoking an order: **within 48 hours**.
For patients under 18 years detained in a general hospital ([Form 6B](#)) the following applies:
- If the Chief Psychiatrist gives approval for an involuntary detained treatment order in a general hospital, the psychiatrist may make an order for a young person up to 14 days.
- Timeframe to notify third parties including a carer, close family member or other personal support person, of order: **within 2 hours**.
- Timeframe to notify third parties of revoking an order: **within 48 hours**.

Continuation order - The psychiatrist may make a continuation order ([Form 6C](#)) for no longer than 28 days for a young person.

Electroconvulsive Therapy and patients under 18 years – CPG 6.2.5 – 6.2.7
page 121

Off-label treatment given to a patient under 18 years who is involuntary, the psychiatrist must record details of the treatment, product description, reasons for the decision in the Medical Record and provide a copy of the record to the Chief Psychiatrist (see [Off-label prescription to a child who is an involuntary patient - Reporting to the Chief Psychiatrist](#)).

Physical Restraint of Mental Health Patients

Staff must consider particular vulnerabilities in terms of patient's obstetric condition, co-morbid medical illness or complication including substance use and re-traumatisation. Staff to consult with relevant medical personnel (obstetrician, anaesthetist, physician, paediatrician) as required, and outlined in the WNHS Guidelines – [“Mental Health Emergency, Management of”](#). This guideline contains Acute Health-related Behavioural Disturbance flowcharts, principles of physical restraint in pregnancy and postpartum, and principles underlying de-escalation techniques.

Observations and Monitoring

- Staff must ensure the patient's physical and mental health is monitored constantly throughout restraint and any risk managed.
- A mental health practitioner or a nurse must be in physical attendance with the patient, constantly observe the women, provide ongoing care, and verbal de-escalation. Observations must include maintaining a clear airway and

ensuring the restraint is not causing pain or undue discomfort and must be documented on relevant form ([Form 10D](#)).

- The person in charge of the unit must ensure that:
 - Appropriate provision is made for basic patient needs during the restraint.
 - Observations and physical examinations are undertaken.
 - The restraint is ceased if the woman states that she cannot breathe or there are any other signs of physical distress.
 - The treating (or duty) psychiatrist is informed of the restraint within 30 minutes of the restraint and will attend the MBU to review and examine the woman as soon as practicable.
 - A medical practitioner is informed of the restraint as soon as possible and within 30 minutes of the restraint being initiated, to enable an examination of the patient.

Examinations

- Staff must ensure no complication of, or deterioration in, the woman's mental or physical condition has occurred due to physical restraint.
- Following all restraints and within 30 minutes of the restraint being initiated a medical practitioner must examine the patient and review available and less restrictive treatment options with the treating psychiatrist (or duty psychiatrist). If the restraint is to be revoked the medical practitioner will complete the required form ([Form 10G](#)). If the restraint is to be extended, the medical practitioner will complete the required form ([Form 10E](#)) for up to a maximum of 30 minutes.
- Within 6 hours of a restraint ending, a medical practitioner should conduct an examination of the woman to ensure that she has not suffered any complication or deterioration in her mental or physical state ([Form 10I](#)).

After Restraint

- A post-restraint conversation must be completed with the patient by a member of staff of the patient's choice at the first appropriate opportunity. With her consent, her partner/support person should be included in the review process.
- The purpose of the conversation is to clarify events and repair and restore the relationship with staff, as well as to inform the patient's ongoing management in collaboration with the patient, staff and partner/support person.

CPG Standard 6: Seclusion and bodily restraint reduction, page 210 – 212

Office of Chief Psychiatrist –

[Chief Psychiatrist's Standards for Clinical Care](#), pages 19-21

[Reporting Episodes of Seclusion & Restraint](#)

[Form 10 A – I](#) Restraint

Rights of Carer, Close Family Member and Other Support Persons CPG 1.22 pages 48 – 52 and Nominated person CPG 1.15 page 41- 42

Search and Seizure CPG 4.11 page 102

Transport and Transfer Mental Health Patients

Depending on patient condition, transport between MBU and main hospital can be either by ambulance or escorted across by foot.

See the North Metropolitan Health Service [Mental Health Transport Hub](#) which contains flowchart, resources and guidelines.

Transport orders CPG 3.10 page 83, 84

Transfer between authorised hospitals CPG 4.7 page 96

Detention – to enable a person to be taken to an authorised hospital or other place
CPG 3.8 page 77

[Mandatory policy 0063/17](#)

Related WNHS policies, procedures and guidelines

Mother Baby Unit – see procedures

Department Psychological Medicine – see procedures

Guideline – [Mental Health Emergency, Management of](#)

Related legislation, standards and policy

[Mental Health Act \(MHA\) 2014](#)

[Mental Health Regulations 2015](#)

[MH Act 2014 Charter of Mental Health Care Principles.](#)

[Carers Recognition Act 2004](#)

[North Metropolitan Health Service Carers Recognition Policy](#)

[Mandatory Reporting of Notifiable Incidents to the Chief Psychiatrist](#)

[Department of Health Missing Person Policy MP 0012/16](#)

[NMHS Policy: Clinical Incident Management](#)

[NMHS Policy: Incident/Hazard Reporting and Investigation](#)

[NMHS Occupational Safety and Health Hazard/Incident Form](#)

[Responding to an Allegation of Sexual Assault Disclosed within a Public Mental Health Service](#)

[NMHS Mental Health Mental Health Transport Hub](#)

References

Chief Psychiatrist's Delegation Schedules:

- [Approval for Inpatient Treatment Order in General Hospital](#)
- [Change in Supervising Psychiatrist for a Community Treatment Order](#)
- [Approval for emergency electroconvulsive treatment.](#)

[Clinician's Practice Guide to the Mental Health Act 2014 3rd Edition](#)

[Off-label prescription to a child who is an involuntary patient - Reporting to the Chief Psychiatrist](#)

[Mental Health Advocacy Service.](#)

[Mental Health Commission Mental Health Act 2014 resources](#)

[Mental Health Law Centre](#)

MP 0063/17 [WA Health Requesting transport officers and WA police assistance in transporting mental health patients Policy](#)

MP 0060/17 [Use of Physical and/or Mechanical Restraint during Transportation of Mental Health Patients Policy](#)

Useful resources (including related forms)

[Helping Someone in Distress - MHC booklet 2017](#) – booklet

[Information for Personal Support Persons - MHC booklet 2017](#)

[Nominated Persons - MHC booklet 2017](#)

[MHA 2014 Forms \(HealthPoint\)](#)

[MHA 2014 Forms writable and printable](#) (Office of Chief Psychiatrist)

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The health impact upon Aboriginal people has been considered, and where relevant incorporated and appropriately addressed in the development of this policy (ISD Number - 528).

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